

Sexual revictimisation risk mitigation among survivors of sexual violence and trafficking: A multi-method feasibility study of a body-oriented treatment module

R. S. Ghafoerkhan, W. F. Scholte, S. Bieleveldt, N. van der Aa & P. A. Boelen

To cite this article: R. S. Ghafoerkhan, W. F. Scholte, S. Bieleveldt, N. van der Aa & P. A. Boelen (2025) Sexual revictimisation risk mitigation among survivors of sexual violence and trafficking: A multi-method feasibility study of a body-oriented treatment module, *Counselling Psychology Quarterly*, 38:3, 479-506, DOI: [10.1080/09515070.2024.2424160](https://doi.org/10.1080/09515070.2024.2424160)

To link to this article: <https://doi.org/10.1080/09515070.2024.2424160>



© 2024 The Author(s). Published by Informa UK Limited, trading as Taylor & Francis Group.



[View supplementary material](#)



Published online: 23 Nov 2024.



[Submit your article to this journal](#)



Article views: 1969



[View related articles](#)



[View Crossmark data](#)



OPEN ACCESS



Sexual revictimisation risk mitigation among survivors of sexual violence and trafficking: A multi-method feasibility study of a body-oriented treatment module

R. S. Ghafoerkhan^{a,b}, W. F. Scholte^{c,d}, S. Bieleveldt^e, N. van der Aa^a and P. A. Boelen^{a,b}

^aARQ National Psychotrauma Centre, Diemen, The Netherlands; ^bDepartment of Clinical Psychology, Utrecht University, Utrecht, The Netherlands; ^cLaguna Collective, Amsterdam, The Netherlands; ^dDepartment of Psychiatry, Amsterdam UMC, University of Amsterdam, Amsterdam, The Netherlands; ^eAdagio Mental Healthcare, Amsterdam, The Netherlands

ABSTRACT

Background: The majority of sexual violence survivors become sexually revictimised, and migrant populations are at particular risk. Prior research indicates that limited body awareness, dissociation, and difficulties in communicating boundaries might be underlying mechanisms of sexual revictimisation. Mental healthcare disregarding sexual revictimisation may insufficiently address the population's needs.

Aims: In the current study, a novel individually delivered body-oriented module was considered for its feasibility amongst migrant survivors of sexual violence and trafficking.

Method: Thirteen migrant sexual violence survivors engaged in the module in an outpatient mental health care setting in the Netherlands. A multi-method approach was applied to tap into patients' and therapists' perspectives on the feasibility of the module using pre-, during, and posttreatment questionnaires, evaluation forms, and (group) interviews.

Results: Patients and therapists reported treatment outcomes by the module's aims, and treatment adherence was high. Bayesian informative hypothesis evaluation revealed that, for seven out of 12 patients (58.33%), body awareness, and self-efficacy in communicating boundaries increased, while body dissociation simultaneously decreased during treatment.

Discussion: Findings suggest that a body-oriented module for sexual revictimisation risk mitigation is feasible amongst migrant survivors. Given the small sample size and lack of a control group, the findings should be interpreted cautiously.

PRACTICAL IMPLICATIONS

- Mental healthcare should consider the risk of sexual revictimisation, especially for forced migrant populations.
- Body-oriented approaches may be feasible in mitigating sexual revictimisation amongst forced migrant survivors.
- Increasing body awareness, self-efficacy in communicating boundaries and reducing dissociation may be key elements in understanding sexual revictimisation risks.

ARTICLE HISTORY

Received 9 November 2023
Accepted 29 October 2024

KEYWORDS

Forced migration; sex trafficking; sexual violence; sexual revictimisation; mental health; body-oriented therapy; informative hypothesis testing

CONTACT R. S. Ghafoerkhan  r.ghafoerkhan@arq.org

© 2024 The Author(s). Published by Informa UK Limited, trading as Taylor & Francis Group.

This is an Open Access article distributed under the terms of the Creative Commons Attribution License (<http://creativecommons.org/licenses/by/4.0/>), which permits unrestricted use, distribution, and reproduction in any medium, provided the original work is properly cited. The terms on which this article has been published allow the posting of the Accepted Manuscript in a repository by the author(s) or with their consent.

The majority of sexual violence survivors will experience sexual violence again later in life (Classen et al., 2005; Walker et al., 2019). This phenomenon, referred to as “sexual revictimisation”, raises the question of how to understand and address this elevated risk. Yet, the risk of sexual revictimisation has received little attention within mental healthcare, especially for migrant populations (Baba et al., 2023). In the present study, we introduce and evaluate a novel body-oriented treatment module aimed at mitigating the risk of sexual revictimisation among survivors. We chose to focus on traumatised migrant populations in the Netherlands, including refugees, asylum seekers, trafficked people, and first and second-generation migrants from former Dutch colonies. This population is at high risk for revictimisation (Rubini et al., 2023), while there is limited research available on how to address the underlying mechanisms of this risk.

Addressing sexual revictimisation among migrant populations

Worldwide, the number of people who are forced to leave their homes due to war, conflict, and prosecution is growing (United Nations High Commissioner for Refugees, 2023). Many of these migrants may have experienced sexual violence or trafficking in their home country, during their journey to safety or in host countries (Boskovic & Jankovic, 2023; Latham-Sprinkle et al., 2019; Tol et al., 2013). Migrant populations are at particularly high risk for sexual (re-)victimisation and (re-)trafficking, yet research on this population is scarce (David et al., 2019; International Organization for Migration, 2019; National Rapporteur on Trafficking in Human Beings and Sexual Violence against Children, 2021). The risk of sexual violence may be explained by the loss of social fabric in times of conflict (Scholte & Ager, 2014) or may intersect with one’s sexual orientation and gender identity expression (SOGIE; Nilsson et al., 2021). Research indicates that during migration, women, girls, and those with diverse SOGIEs are disproportionately targeted by traffickers (Buckinx et al., 2022; Vu et al., 2014).

Prevalence rates of sexual revictimisation are challenging to establish, especially for male survivors (De Schrijver et al., 2018; International Organization for Migration, 2019; Keygnaert et al., 2012), given that the number of migrants who seek mental healthcare is disproportionately low (Byrow et al., 2020). Nonetheless, research indicates that one in five refugees or displaced women in humanitarian settings experienced sexual violence (Vu et al., 2014). For migrants residing in Europe, the risk of sexual revictimisation is often associated with a lower socioeconomic status, resulting in limited financial resources, unawareness of legal rights, or feeling unsafe to approach protective services (Baba et al., 2023; International Organization for Migration, 2019; Rich et al., 2004). Furthermore, asylum seeker centres, often overcrowded and with sanitary provisions insufficiently securing privacy, can be unsafe environments. Persons from varying backgrounds may have to live together in confined spaces, which easily elicits tension and violence. In addition, people from diverse SOGIE may experience discrimination and intimidation against them (Nilsson et al., 2021). Such factors exacerbate the fear and risk of sexual violence and revictimisation (Keygnaert et al., 2012) and warrant the attention of mental health-care providers. Support that disregards these realities may lead to insufficiently

addressing the population's needs, and this underscores the importance of treatment tailored to migrants' realities.

The psychological aftermath of sexual violence

Research indicates a high prevalence rate of mental health disorders among migrant populations, such as asylum seekers, refugees, and survivors of trafficking (Blackmore et al., 2020; García-Vázquez & Meneses-Falcón, 2023). In a study among refugees in high-income countries, prevalence rates were found to be high even when compared to other populations living in conflict or war settings (Henkelmann et al., 2020). Next to this, the aftermath of sexual violence is linked to a wide range of negative short and long term mental health outcomes among migrants (Dworkin, 2020; Oram, 2019; Ottisova et al., 2016), such as (complex) post-traumatic stress disorder (PTSD; Dworkin et al., 2023).

Sexual violence is a gross violation of one's dignity, personal space, and physical boundaries (Foundation, 2019). Beyond PTSD complaints that need to be addressed in evidence-based psychotherapies, survivors may face difficulties in their overall quality of life, including their interpersonal and sexual functioning (Mills & Turnbull, 2004; Rothman et al., 2021). Some survivors may become submissive while discounting their own needs, whereas others may become over-assertive with a fixed control over their boundaries (Katz et al., 2010; Livingston et al., 2007; Macy, 2007). There are several factors that may compound these psychological sequelae. For instance, when the sexual violence starts at a young age (Downing et al., 2021) or lasts over an extended period, as in the case of sex trafficking (Abas et al., 2013). These sequelae may also be compounded when sexual violence is deeply embedded in existing power dynamics in one's community, like marginalisation based on gender, ethnicity, or SOGIE (Buss, 2009; Flanders et al., 2023). Next to a focus on reducing PTSD symptoms within mental healthcare, it is thus crucial to address the impact on interpersonal functioning and revictimisation as well.,

Underlying mechanisms for sexual revictimisation

Research highlights sexual violence's psychological aftermath, such as the impact on body awareness and dissociation, as a potential underlying mechanism for sexual revictimisation (Risser et al., 2006; Scoglio et al., 2021; Ullman & Vasquez, 2015; Walker & Wamser-Nanney, 2023). Body awareness refers to the ability to experience, notice, and identify inner bodily sensations and needs, and body dissociation refers to a disconnection from these sensations and needs (Price et al., 2017). For instance, disconnecting from one's bodily signs can serve as a survival mechanism by numbing sexual violence's severe impact (Schauer & Elbert, 2010). This response may be encoded into memory records of the event and may be activated at present when the survivor's perceived physical integrity is being threatened (Neuner, 2023). When physical and personal boundaries are continuously violated, this post-traumatic reaction may continue and become maladaptive. In these cases, one might become disconnected from noticing bodily cues, needs, emotional states, boundaries, or intuition (Burback et al., 2023; Kearney & Lanius, 2022; Messman-Moore & Brown, 2006; Schimmenti & Caretti, 2016). Prior research indicates that lack of body awareness or dissociation may

elevate the risk of sexual revictimisation (Risser et al., 2006; Walker & Wamser-Nanney, 2023). For instance, someone may be unable to notice or act on early warning signs of boundaries being crossed in harmful situations (Breitenbecher, 2001; Noll & Grych, 2011). Additionally, one might have unlearned or never had the opportunity to learn how to effectively identify and communicate one's personal boundaries (Livingston et al., 2007). As suggested by prior research, this lack of body awareness or dissociation may contribute to sexual revictimisation risk in sexual violence survivors (Bockers et al., 2014; Katz et al., 2010; Kelley et al., 2016).

Addressing the risk of sexual revictimisation

The module evaluated in the present study focussed on increasing body awareness and communication of personal boundaries and decreasing body dissociation. Staying safe and breaking patterns of victimhood may be best addressed within a body-oriented approach. These modules utilise movement, role-play, and bodily experiences to address the aftermath of trauma (O'Brien & Charura, 2022; Schaeffer & Cornelius-White, 2021; Van De Kamp et al., 2023). Such body-oriented approaches have not yet been validated amongst migrant populations. Treatment modules focussed on reducing the risk of sexual revictimisation offer promising outcomes, and their content suggests potential underlying mechanisms of revictimisation (e.g. Messman-Moore & McConnell, 2018; Wolfe, 1996). However, to the best of our knowledge, an evidence base for modules specifically targeting migrant survivors of severe or chronic sexual violence is currently missing (e.g. Robitz et al., 2022). Various factors complicate mental healthcare provision in the aftermath of sexual violence against migrants. In general, disclosure of sexual violence is infused with shame and stigma (Bhuptani & Messman-Moore, 2019). This may be particularly the case for migrant populations, due to cultural notions of sexual integrity (Tankink & Richters, 2007). Furthermore, some migrants are uncertain about their residential status and access to shelter, healthcare, and allowance (Li et al., 2016). Such daily stressors complicate committing to and regularly attending long-term therapies (Byrow et al., 2020; Semmlinger & Ehring, 2022), suggesting short-term therapies may encourage treatment feasibility amongst this population.

The current study

The module evaluated in this study was developed based on the research above and clinical experience at the study site. The module was targeted specifically towards first- and second-generation migrants resettled in the Netherlands, who were survivors of severe/chronic forms of sexual violence for instance, sex trafficking or conflict-related and prolonged childhood sexual abuse. The module highlights four themes using a body-oriented approach: i) (re-)connecting to bodily cues/sensations and emotional states; ii) recognising and expressing personal boundaries; iii) identifying potentially unsafe situations; iv) discussing potentially harmful gender norms and responding to potentially unsafe situations.

Aims

The overarching aim of this study was to examine if patients and therapists considered the module feasible. In the present study, feasibility was defined by whether therapists and patients could adhere to and complete the module's protocol, as well as whether both groups found the session content and treatment outcomes relevant for mitigating the risk of sexual revictimisation. The following indices were considered (1) treatment adherence rates, therapists' session evaluations, and completion rates; (2) patients' pretreatment expectations and posttreatment evaluations of the module; (3) increase in patients' scores on body awareness, decrease in body dissociation, and increase in self-efficacy in communicating boundaries during the module.; and (4) therapists' reflections on the module. Finally, a fictitious case was presented to offer insight into the target population and treatment process.

Method

Setting and patients

This study took place at an outpatient mental health clinic based in the capital region of the Netherlands. In this clinic, one multidisciplinary team specifically serves people with a migrant background who have faced sexual violence and/or sex trafficking. In some cases, these experiences intersect with violence related to their SOGIE. Upon referral by a general practitioner, psychodiagnostics and treatment are offered, aimed at PTSD symptom reduction and personal recovery.

The inclusion period ran between February 2020 and September 2022. Male and female patients (≥ 18 years) within the clinic were considered for inclusion via purposive sampling if they: i) had a history of sex trafficking or severe/chronic sexual violence; ii) were deemed vulnerable to sexual revictimisation as expressed by themselves or as estimated during a multidisciplinary team meeting; iii) gave informed consent for participation in the module and administration of the additional measures and interviews, and allowed researchers access to their medical records. Patients displaying acute crises, such as acute severe psychosis, persistent substance abuse, or acute suicidality, were excluded.

Design and ethical approval

A multi-method approach was applied amongst patients and facilitators using pre-, during and posttreatment questionnaires, evaluation forms, and (group) interviews. Ethical approval for this study was obtained from the Medical Ethics Committee Leiden The Hague Delft in the Netherlands (P17.270). The study was pre-registered at the Dutch Trial register (NL61808.058.17: https://www.toetsingonline.nl/to/ccmo_search.nsf/fABRpop?readform&unids=D707FF3CE1AFE7D3C125881F00152BB7,NTR7353, July 11th 2018). Additional ethical approval for the group discussion was obtained from the Ethics Review Board of the Faculty of Social and Behavioural Sciences at Utrecht University (21–0460).

Intervention

The individually delivered module and its core themes and exercises were developed by a psychologist (author RG) with over 10 years of experience in working with survivors of sexual violence and a psychomotor therapist (author SB) with over 20 years of experience in working with survivors of sexual violence. A psychomotor therapist offers experiential, body, and movement-oriented interventions for people with mental health problems. These interventions help individuals explore, become aware of, acknowledge, and change their sensations, emotions, thoughts, and behaviours while also practicing new behaviours. The session plans of the module were based on vast clinical experience at the study site and a non-systematic literature review. This review entailed a web-based search of existing modules in scientific journals and manuals published by health- and youth care institutes. The module was mainly embedded in principles of sensorimotor therapy (Ogden, 2015), the theory on the impact of trauma on the body (Van der Kolk, 2015), theory on the role of dissociation after trauma (Schauer & Elbert, 2010), existing knowledge around complex PTSD (Farrell & Shaw, 1994; Maercker et al., 2022), and the sexual revictimisation prevention program by Wolfe (1996). In addition, the module was reviewed in-depth by various clinicians at the study site and by two former patients as experts with lived experience of migration, sexual violence, and/or trafficking and receiving mental healthcare.

The module was offered by a clinical team specialised in sexual violence among migrant populations. A total of eight female therapists provided the module: five psychologists, two psychomotor/movement therapists, and one social worker. The therapists had between 1 and 20 years of experience in working with survivors of sexual violence. An eight-hour training session was provided by the primary developers (authors RG and SB), with the support of an expert with lived experience. The module was carried out individually each time by one therapist over 10 sessions of 1 hour. Translators, either in-person, via video calling, or via telephone, were available when needed, and materials were available in Dutch, English, and French.

The module had a body-oriented approach with elements of psycho-education and reflective conversation. The main aims of the module were to promote body awareness, mitigate body dissociation, and offer tools to encourage efficacy in communicating boundaries in potentially unsafe situations. Therapists were experienced in dealing with trauma-related symptoms. However, it was clearly conveyed to the patients that the module was not trauma-focussed or aimed at processing past traumatic experiences. The module was specifically tailored to the needs of (forced) migrants. For instance, the vignettes were modified to situations applicable to those living in an asylum seeker shelter. See Table 1 for a brief overview of each session plan. Next, to the introductory and closing sessions, the module contained four core themes, each addressed in two consecutive sessions. The first theme, “feeling”, was focussed on (re-)connecting to bodily cues/sensations and (re-)gaining awareness of emotional states. The second theme, “autonomy”, centred around recognising and expressing personal boundaries. The third theme, “safety”, was about identifying and responding to potentially unsafe situations. Finally, the fourth theme, “empowerment”, introduced the topic of potentially harmful and suppressing gender roles and potentially unsafe situations in one’s daily life specifically.

Table 1. Brief overview of the session plans.

Session and theme	Aims	Topics and exercises
1. Introduction	Introducing core elements of module. Establishing rapport and therapeutic alliance.	Emotion regulation and self-regulation. Establishing safety measures during module.
2. Feeling	Distinguishing (un)pleasant body sensations. Recognising tension/relaxation in the body.	Sensory exercises. Relaxation exercises.
3. Feeling	Recognising and embodying feelings of (un) safety.	Creating a safe space in the room.
4. Autonomy	Recognising non-verbal communication. Learning to express personal boundaries.	Walking exercises. Voice and body language exercises.
5. Autonomy	Exploring grounded standing and resilience.	Grounded standing. Personal space and controlled approach exercises.
6. Safety	Identifying and responding to potentially unsafe situations	Vignettes containing potentially unsafe situations. Psycho-education on sexual revictimisation.
7. Safety	Identifying and responding to potentially unsafe situations in one's daily life. Recognising and embodying feelings of safety (repetition).	Reflective conversation and role-play on a current unsafe situation in one's daily life.
8. Empowerment	Exploring one's own perspective on gender roles and (romantic) relationships.	Reflective conversation on gender roles. Body posture exercises.
9. Empowerment	Practicing setting boundaries in potentially unsafe situations in one's daily life.	Role-play with vignettes and current unsafe situations in one's daily life.
10. Closing	Exploring how to further integrate what has been learned in one's daily life.	Reflection and saying goodbye

Measures

Therapists

The following information was derived from the session records: the total number of sessions delivered, total numbers of and reasons for cancellations/no-shows, total treatment duration, and reasons for drop-out.

Evaluation form. After each session, therapists filled out an evaluation form to indicate whether or not they had completed all the elements as described in the module (“yes” or “no”), and if not, to indicate the reason. Second, to respond to the following statements from their personal perspective using a visual analogue scale (VAS, 0 = “not at all” to 100 = “very much”): “There was enough time for the session plan”; “The session was a burden to the patient”; “I noticed personal resistance or a lack of trust in the session plan”; “I felt energised/I looked forward to carrying out the session plan together”; and “I felt the session was useful for the patient”.

Group discussion with therapists. At the end of the inclusion period a group discussion was held with the therapists to evaluate the module. The topic guide for the discussion included the following questions: “What worked well in the delivery, content, and outcomes of the module?”; “What could be improved about the delivery, content, and preferred outcomes of the module?”; “Did you miss a particular subject or exercise?”; “The module introduced several concepts that may contribute to the risk of sexual revictimisation. This included body awareness, dissociation, and self-efficacy in communicating boundaries. What did you think of these topics, did they resonate for your

patient?"; "Do you believe that by partaking in the module the patient has become less vulnerable to sexual revictimisation?".

Patients

Semi-structured interview with patients. Before starting the first session of the module (pretreatment), a first short semi-structured interview was held with the patients. After participation in the last session of the module (posttreatment), a second short semi-structured interview was held with patients. During pretreatment, patients were asked: "What do you think the module is about?" and "What do you expect to learn during the module?". At posttreatment, patients were asked: "What did you learn during the module?", "What were important topics and/or exercises?", and "What were topics and/or exercises you did not like or did not find useful?".

Questionnaires. Main outcomes of interest, i.e. body awareness, body dissociation, and self-efficacy in communicating boundaries, were measured at the start of each session on a VAS (0 = "not at all" to 100 = "very much"). Given time constraints and to minimise the burden on the participants, one item per outcome was selected from the full-scale measures described below. Items that best aligned with the intended treatment outcomes were carefully selected from the full scales through close collaboration and deliberation among the team of authors and clinical team. In addition, prior to the start of the module (pretreatment) and after participation in the module (posttreatment), several full-scale measures were administered for descriptive purposes.

Scale of body connection (SBC). This scale measures body awareness and body dissociation in 20 items scored on a 5-point scale (0 = "not at all" to 4 = "all of the time"). As indicated by previous studies, the full scale should not be considered, but rather its two distinct subscales: 1) body awareness (BA; 12 items) and 2) body dissociation (BD; 8 items; Price & Thompson, 2007; Price et al., 2017). Both subscales were found to have good psychometric properties in undergraduate students (Price & Thompson, 2007; Price et al., 2017). For the BA subscale, a higher score indicates more awareness, and for the BD subscale, a lower score indicates less body dissociation. The two subscales were administered at pre- and posttreatment for descriptive purposes of the sample. Two items in line with the aims of the module were chosen to be administered prior to each session. From the BA subscale: "I listen for information from my body about my emotional state", and from the BD subscale: "My body feels frozen, as though numb, during uncomfortable situations." The validated English version of these subscales (Price & Thompson, 2007) was used. For the Dutch version, only the subscale BA has been psychometrically evaluated (Van der Maas et al., 2014) and was used. Unfortunately, the Dutch version of the BD has not been psychometrically evaluated.

Sexual assault resistance self-efficacy (SA-RSE). This scale was developed to assess the perceived self-efficacy in response to communicating boundaries in a hypothetical unwanted sexual advance (Littleton & Decker, 2017). This scale was found to have adequate psychometric properties and to be able to predict sexual revictimisation amongst a sample of women with a rape history (Littleton & Decker, 2017). For this study, one item was selected from the original 16-item self-report scale on a 5-point

scale to match the module's aims: "If someone I know forces me to do something I do not want to do, I feel confident to stop this person.", scored on a VAS (0 = "not at all" to 100 = "very much"). Higher scores indicate more self-efficacy in communicating boundaries. This single item was administered pre- and posttreatment, and before each session. This scale and single item are only available in English, and for the purpose of this study, were back translated into Dutch.

Post-traumatic stress symptoms and traumatic load. The Post-Traumatic Stress Checklist-5 (PCL-5), Life Events Checklist-5 (LEC-5), and Early Trauma Inventory Self Report-Short Form (ETISR-SF) were administered to understand the levels of PTSD symptom severity and the traumatic load within the sample. The PCL-5 assesses the severity of 20 symptoms of PTSD as defined by the DSM-5 on a 5-point scale (U.S. Department of Veterans Affairs, 2022). This measure was administered during pre- and posttreatment, as it is considered suitable for monitoring changes in symptoms with good psychometric properties (U.S. Department of Veterans Affairs, 2022; Van Praag et al., 2020). The summed item scores represent PTSD symptom severity varying between 0 and 80. Research indicates that a cut-off score between 31 and 33 to be indicative of clinically relevant PTSD and a change score of 10 points as a minimum threshold for determining clinically meaningful improvement (U.S. Department of Veterans Affairs, 2022). This checklist was used in validated versions of English (U.S. Department of Veterans Affairs, 2022) and Dutch (Van Praag et al., 2020).

The ETISR-SF and LEC-5 were administered only during pretreatment to establish the traumatic load within the sample. The ETISR-SF assesses potentially traumatising events as experienced before the age of 18 years old (Bremner et al., 2007). In 27 items, the ETISR-SF enquires on a dichotomous scale ("yes" or "no") whether or not someone has lived through general trauma, physical punishment, emotional abuse, or sexual events. The LEC-5 considers the lifetime prevalence of 16 types of potentially traumatising events, in addition to the option for an open-ended response regarding a life event. The respondent is asked to indicate on a 6-point nominal scale whether this event: "happened to me", "witnessed it", "learned about it", "part of my job", "not sure", or "does not apply". The psychometric properties of these measures can be considered good (Bremner et al., 2007). These checklists were used in the validated English versions and were back translated into Dutch at the study site.

Procedure

Based on the study's inclusion criteria, patients' eligibility was determined within a multidisciplinary team meeting and in close collaboration with the primary researchers. After the patient gave informed consent, a pretreatment assessment was performed 1 week or less prior to the first session. The post-treatment assessment took place directly after the last session or at a maximum of 1 week after completion. The questionnaires and interviews were administered in person by either master-level psychology students, junior researchers, or the primary investigator RG. Some measures designed as self-report questionnaires were read aloud by the administrator, given some of the participants' limited vocabulary or reading proficiency. All questionnaires and interview questions were available either in Dutch or English. When needed, a translator was available via telephone. On average, the assessments lasted 82 min for pretreatment and 47 min for

posttreatment (as the ETISR-SF and LEC-5 were not administered post-treatment). Assessments during treatment were administered by the therapists on hardcopy prior to each of the 10 sessions. All data, including minutes taken from the pre- and posttreatment interviews, were entered into a data file anonymously.

At the start of the inclusion period, the COVID-19 pandemic broke out worldwide, and the study site paused to see patients on location. The inclusion period was therefore paused from March 2020 and continued in October 2020. They only included patient discontinued treatment after two sessions and were included again later on. Thereafter, it was sometimes necessary to deviate from the study protocol to conform to the pandemic regulations. For instance, a small number of meetings were cancelled or held hybrid due to testing positive for COVID-19, required self-isolation, or schools being closed.

To monitor the treatment process and identify (potential) obstacles in treatment adherence, researchers and therapists met monthly. Next, researchers and therapists corresponded via email regularly in between the scheduled meetings. Group meetings were also held between therapists to share experiences in applying the module. The module was offered in various languages: Dutch ($n = 5$), English ($n = 5$), French using a translator ($n = 2$), and Lingala using a translator ($n = 1$). During inclusion, parallel interventions were not allowed, apart from ongoing pharmacotherapy or interventions needed in case of acute crisis.

All the therapists ($N = 8$) were invited to participate in the group discussion by the primary investigator (author RG). Two therapists conducted only a few sessions before dropping out of their patients and, therefore, chose not to participate. One therapist was not able to join due to scheduling difficulties. The discussion was consequently held with five therapists. Due to COVID-19 regulations, this meeting took place via video calling, and informed consent was signed digitally. The recorded discussion was transcribed anonymously by a master-level psychologist.

Analytic plan

Qualitative analyses

The transcripts from the pre- and posttreatment patient's semi-structured interviews (aim 2) were read carefully to identify recurring topics patients brought forward. For each patient, core statements from their transcript were selected to be presented in [Table 3](#).

The transcripts and recordings of the therapists' group discussion (Aim 4) were thoroughly read and listened to, followed by a comprehensive coding and thematic analysis based on the approach by Braun and Clarke ([2022](#)). The analysis employed both pre-determined codes and an open-coding process, for creating and refining codes during the analysis. Pre-determined codes were determined by the main study aim of understanding the feasibility of the module. These codes included "observed outcomes aligned with the module's aims", "negative outcomes related to the module", "relevance of selected topics and exercises within the module", and "considerations regarding module delivery".

As the group discussion was analysed, additional codes emerged from the data. These included categories such as "the impact of active exercises," "change mechanisms within the module," "uncertainties about the module's rationale," "contra-indications for participating in the module", and "limitations of the module". Attention was also given to any contradictory

responses, which provided valuable insights into the diversity of therapists' experiences. The coding and thematic analysis process was undertaken by two independent coders, the primary investigator (author RG), and a research assistant (a master-level psychology student). After independent analyses, the coders compared the codes they identified from the data during meetings. Some subsections of the transcripts were discussed in detail to see if coders needed clarification about the message the interviewee wanted to convey.

Quantitative analyses

The scores of self-efficacy in communicating boundaries, bodily awareness, and dissociation (aim 3) during treatment were analysed on an individual level. To gain insight into the course of change in these main outcomes, the scores were plotted as a function of the sessions. To test inferences about the course of the main outcomes, Bayesian informative hypothesis evaluation was used (Hojtink, 2012; Hoijtink et al., 2019). This approach had the advantage that inferences about three different outcomes could simultaneously be incorporated into one informative hypothesis (H1) being tested. We created a model stating that body awareness and self-efficacy increased, whereas body dissociation simultaneously decreased over the course of treatment. This model was tested using an informative hypothesis (H1) against the null hypothesis (H0) stating that the model was not true for the current data. The informative hypothesis (H1) was also tested against the joint complement hypothesis (H2), stating that something else was entirely at hand within the current data. Data were analysed in R Studio version 4.1.1 using the R package *bain* and *restrictor* (Gu et al., 2021; Van Brabant et al., 2023). For each individual patient, the three main outcomes were regressed on the session numbers. The informative hypothesis was evaluated using the Posterior Model Probabilities (PMPc) and the weights resulting from the Generalised Order-Restricted Information Criterion (GORICA). Posterior model probabilities quantify the support in the data for the hypothesis specified (H1), the null hypothesis (H0), and their joint complement to the informative and null hypothesis (H2; Hoijtink et al., 2019). The same holds for the GORICA weights (Altinisk et al., 2021).

Results

Descriptive statistics

A total of 14 patients were included in this study; one patient dropped out after the pretreatment assessment and was excluded from further analyses. See Table 2 for socio-demographic characteristics and an overview of the pre- and posttreatment scores for the full sample ($n = 13$). Notably, scores on the ETISR-SF revealed that other types of traumas before the age of 18 years were highly prevalent within the sample, i.e. mainly childhood physical and emotional abuse, domestic violence, and losing a primary caretaker. All patients were diagnosed with PTSD at the study site via clinical assessment within a multidisciplinary team of registered mental healthcare professionals. At the study site, newly admitted patients are generally assessed by a medical doctor or master-level psychologist under the supervision and responsibility of a registered senior mental healthcare professional, such as a specialised clinical psychologist or psychiatrist. In line with the seniors' clinical observations, assessment, and discussions within a multidisciplinary team meeting, a clinical diagnosis is

Table 2. Sociodemographic characteristics and pre- and posttreatment questionnaires.

	<i>n</i>	%
Gender		
Female	11	79
Male	2	14
Country of origin		
The Netherlands (second generation migrant)	4	29
Uganda	3	21
Guinee	2	14
Democratic Republic of Congo	1	7
Egypt	1	7
Moldova	1	7
Nigeria	1	7
Sexual orientation		
Heterosexual	8	57
Homosexual	5	36
Place of residence		
Specialised shelter for sex trafficking survivors	6	43
Asylum seeker centre	4	29
Independent living	3	21
Traumatic background ^a		
Survivor of sex trafficking	10	71
Forced marriage	4	29
Other forms of sexual violence	3	21

	Pretreatment		Posttreatment	
	<i>M</i>	<i>SD</i>	<i>M</i>	<i>SD</i>
Body awareness	28.77	8.30	30.44	5.96
Body dissociation	19.50	6.07	17.33	5.20
Self-efficacy in communicating boundaries	38.46	31.09	71.22	26.74
Posttraumatic stress symptoms	53.17	13.41	46.50	12.02
Traumatic load (≤ 18 years)	17.55	3.45	-	-
Traumatic load (lifetime)	3.23	1.96	-	-

N = 13. Patients were, on average, 29.62 years old (*SD* = 8.17, range: 19–49).

^aMultiple categories could apply to patients.

given. During pretreatment, all patients but one scored clinically on the PCL-5 ($M = 53.17$, $SD = 13.41$, range: 25–76). Although the module was not aimed toward PTSD symptom reduction or the processing of traumatic memories, three out of nine completers showed a clinically meaningful decrease in PTSD symptom severity from pre- to posttreatment; four showed a minor decrease or increased severity; for one, patient data were missing.

Treatment adherence and completion (aim 1)

See [Figure 1](#) for the flow diagram of the study. Nine patients completed the treatment (64% of the sample) and four discontinued treatment (29% of the sample, between 2 and 8 meetings). For those who completed the module, the total length of treatment in days was 120 days ($SD = 42$). In general, treatment adherence was high; out of the completers, two patients had one or two sessions where the therapist deviated from the session plan, mainly due to acute daily stressors or time constraints. Likewise, the evaluation form completed by therapists on VAS after each session indicated a high level of adherence ($M = 90.31$, $SD = 8.70$). In addition, therapists experienced having too little time to implement all the elements of the session plan ($M = 52.04$, $SD = 11.90$), found the module quite a burden to the patients ($M = 38.18$, $SD = 9.92$), viewed the session content as useful ($M = 73.29$, $SD = 6.56$), and finally the therapists felt little resistance ($M = 24.44$, $SD = 9.67$) and energised ($M = 73.18$, $SD = 10.18$) when delivering the sessions.

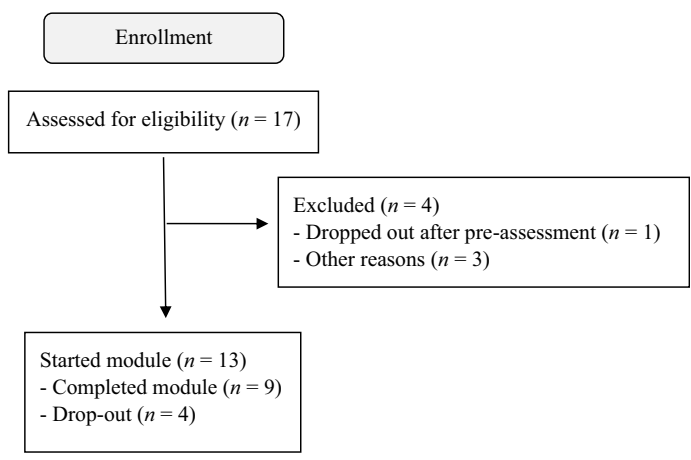


Figure 1. Flow diagram study, adapted from CONSORT 2010 flow diagram (Schulz et al., 2010).

Patients’ pretreatment expectations and posttreatment evaluation (aim 2)

Table 3 provides an overview of the total number of sessions, reasons for discontinuation, and summaries of the pre- and posttreatment semi-structured interviews per patient. Some patients (B, C, and G) had difficulty in formulating expectations of what they hoped to gain during the module. For instance, one patient, before starting the module, said: “I do not know exactly what it is about; this is my first time receiving such a treatment”. Others hoped to process their past/traumatic experiences (patients D, E, and M), learn how to set boundaries or say “no” (patients A, F, and I), regulate emotions or themselves (patients B, L, and J), or to trust other people (patient C). One patient shared beforehand: “I want to understand myself better and become more confident in saying ‘no’ to things that feel wrong”.

In posttreatment, when probed for what they had learnt during the module, patients reported a wide range of outcomes. Many expressed learning about boundaries, how to say “no” or to stop someone (patients A, B, C, G, L, and M). One patient illustrated this by giving an example: “I learned I am able to stop someone by using my voice”. Others indicated they had learnt how to assess a situation for its risks and thus prevent potential harm (patients A and F), had re-evaluated/processed their past (patients A and M), had become more aware of their emotions, feelings, and body (patients B, F, L, and M), or experienced more positive emotions (patients C and M). One patient shared “Most importantly, my feelings have returned, and that was the most significant thing for me, especially since they had been shut off for a long time. I noticed my emotions gradually returning”. Finally, one patient felt little hope for her future and had difficulty in formulating what was learned during the module (patient G). There were some exercises that various patients disliked addressing feelings of safety (patient D), expressing boundaries (patient F), the controlled approach exercise during which the patient is in control of the therapist walking slowly towards them (patient L), and finally, one patient disliked talking out loud about the personal life (patient B). One patient shared “I was less interested in the exercise about learning to stand up for myself. That was not the issue I wanted to work on, as I am already good at advocating for myself”.

Table 3. Overview of patients' pretreatment expectations and posttreatment evaluation ($N = 13$).

Patient	Number of sessions	Summary pretreatment interview	Drop-out reason	Summary posttreatment interview
A	10	Understand myself better, learn how to say "no" when things feel wrong.	–	You start to think about your past and what to do to prevent it from happening again. It influences the choices you make, both positive as well as negative choices. I learned to use my voice, I learned I can stop someone with my voice when something happens. I liked the safe place exercise.
B	10	Deal with my emotions I guess. It's therapy, so I will go. . . I hope to learn a lot from it. . . I don't know, these questions are too much for me.	–	I learned to listen to my body and to have the courage to say "no". I liked the safe place, the relaxation exercises, grounded standing and personal boundaries. I did not like to talk about my personal life out loud.
C	10	I don't know what I will learn, this treatment is my first time. I understood it can help me trust other people.	–	I believe in myself. I have the courage to say "no" to someone. So "no" is "no" and "yes" is "yes". I am able to laugh, the module gave me positive feelings.
D	10	I think this treatment is about my past and my traumas. I hope to learn how to deal with it better. Other than that I do not know, that is the way I will approach it.	–	I learned what a posture communicates and how you are perceived by others. I did not like the safe place exercise, it was not useful.
E	10	To put my past to rest, to move forward and don't remain stuck in the past. I hope this is what it will bring me.	–	I learned to be strong during the empowerment exercises.
F	10	For women who have been in my situation, who have been a victims of human trafficking, to learn how to recognise certain types [of people] and avoid them.	–	I learned to estimate situations better, to have more self-confidence in knowing what is normal and what is not. My feelings came back, they had been shut down for a while. I have become more aware of my feelings, to experience and observe them, rather than to immediately act on them. I did not like the exercise where I had to stand up for myself, it is something I already know how to do.
G	10	I don't know. I cannot move on with life. The thoughts will not go away.	–	I do not see a future for myself and I have no hope, because of what I have lived through. I was nice to talk to the therapist and visit my safe place. I learned to say no.

(Continued)

Table 3. (Continued).

Patient	Number of sessions	Summary pretreatment interview	Drop-out reason	Summary posttreatment interview
H	5	I want to be a happy person again. I don't know what I hope to learn.	Acute psychiatric crisis outweighing the aims of the module.	–
I	2	I lost confidence to react. I hope to gain the confidence to say “no” without any explanation.	Difficulties in arranging childcare.	–
J	8	I hope to learn to become strong just as others. I also hope to be able to talk to others without them becoming angry. I don't want to get angry and hurt others.	Increased psychological distress due to a negative refugee application.	–
K	6	Missing data.	Too many daily/family-related stressors.	–
L	10	It's about safety, I want to be more stable. Not be a mess.	–	Allowing myself to feel what happened, accepting what happened, not avoiding. Not falling into the black hole, embracing the hole and feel everything. I learned to say “no”. It was the first time I realised I dissociate, and it is not normal. The controlled approach exercise reminded me of being physically abused as a child.
M	10	Processing, learn how to deal with it all and leave it behind.	–	At the moment I am more positive. I was able to process things and leave it behind. I liked the personal space exercise. I learned about emotions and relaxation. I learned that it is OK to say what I do not like. I learned about boundaries, before I was always afraid of offending people by saying no.

Patients’ scores on body awareness, body dissociation, and self-efficacy in communicating boundaries during treatment (aim 3)

Figure 2 displays the individual plots of the outcome measures as a function of the sessions, i.e. body awareness, body dissociation, and self-efficacy in communicating boundaries. To allow for a trajectory of change to be evaluated, only those who followed five or more sessions were considered ($n = 12$). The individual plots revealed that some patients’ scores during treatment on body awareness increased over time as hypothesised (Patients C, D, E, F, K, and L), whereas others decreased or did not show any changes (Patients A, B, G, H, J, and M). Self-efficacy in communicating boundaries was expected to increase, which was the case for some (Patients C, D, E, K, L) but not

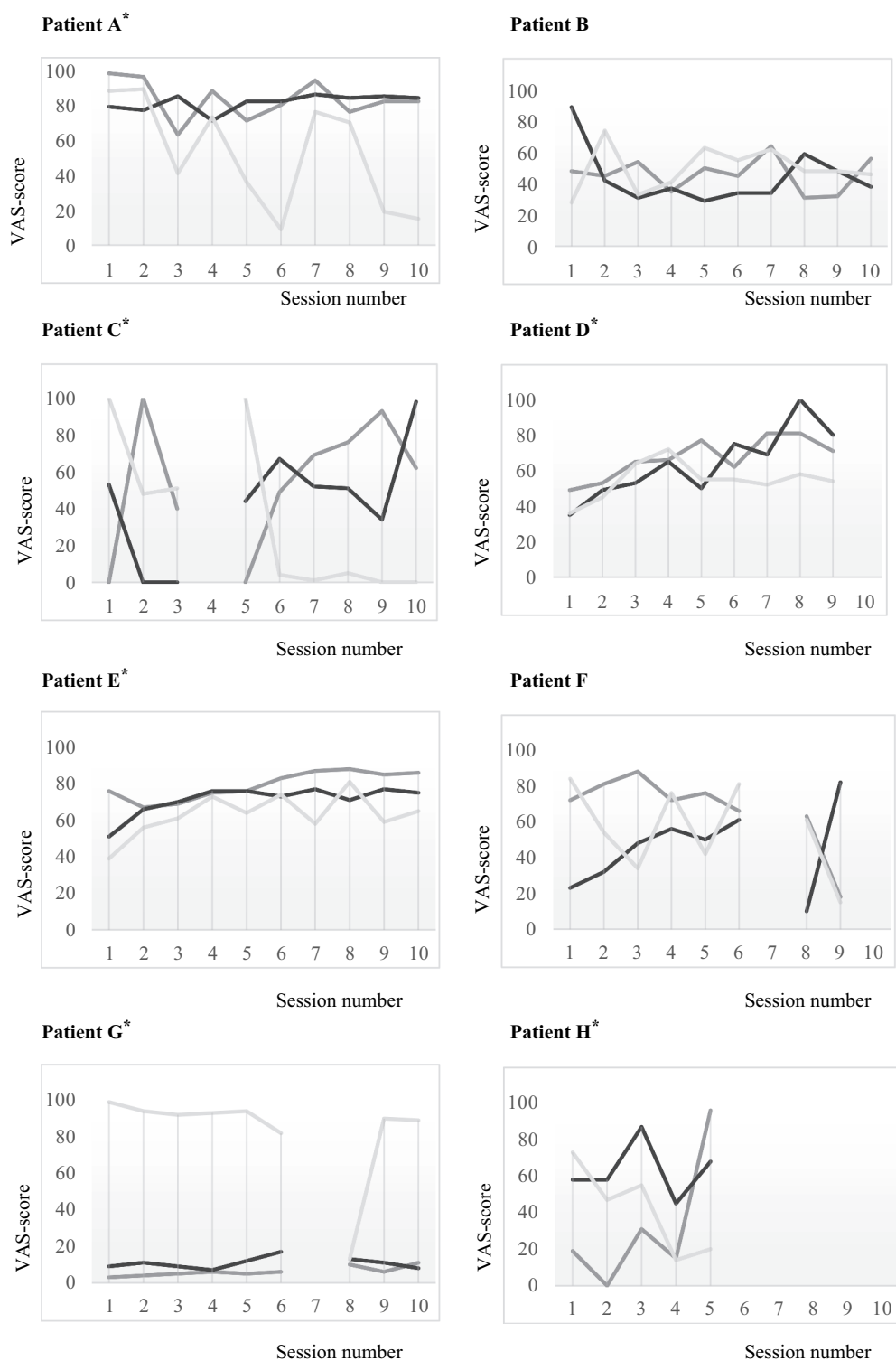


Figure 2a. Individual Scores on Weekly Measures ($n = 12$). The module aimed towards higher scores of body awareness and self-efficacy in communicating boundaries, and lower scores in body dissociation over the course of treatment. *Informative hypothesis supported.

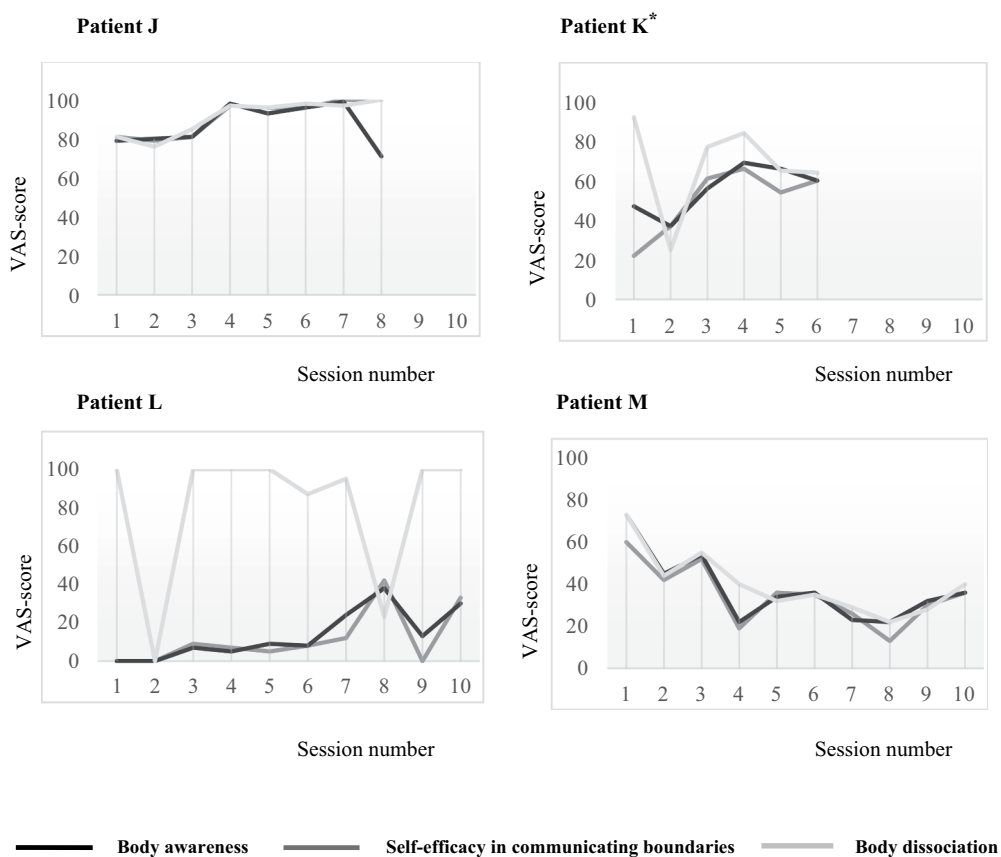


Figure 2. (Continued).

all (Patients A, B, F, G, H, J, and M). Finally, the module aimed to reduce body dissociation, and indeed, a decrease can be observed in some plots (Patients A, C, F, H, K, L, M) but not all (Patients B, D, E, G, J). Using posterior model probabilities and the GORICA weights, the informative hypothesis stated a simultaneous increase in body awareness and self-efficacy in communicating boundaries, with a co-occurring decrease in body dissociation. This hypothesis was tested per patient against the hypothesis (H0; i.e. stating that none of these outcome measures changed over the course of treatment) and against the joint complement of the informative and null hypothesis (H2).

As depicted with an asterisk in Table 4, the PMPc of the Bayes Factor and the GORICA weights indicated that for seven out of 12 patients (58.33% of the total sample) the informative hypothesis received substantial support, whereas the support for the other hypotheses was relatively small. Although analyses indicated that patient G's scores during treatment changed as anticipated, the clinical relevance of these findings might be questionable. The individual plot of patient G (Figure 2) shows that although outcomes change by the informative hypothesis, final scores remain unfavourable clinically.

For three patients (F, J, and M), the results supported the complement hypotheses; another model fits the data better. The individual plots reveal that this might be due to

Table 4. Overview of PMPs and Gorica weights ($n = 12$).

Patient	Informative Hypothesis (H1)		Null Hypotheses (H0)		Joint complement of the informative and null hypothesis (H2)	
	PMPc	GORICA weight	PMPc	GORICA weight	PMPc	GORICA weight
A*	.97	.86	.00	.00	.03	.14
B	.04	.17	.73	.77	.22	.06
C*	.98	.81	.00	.00	.02	.19
D*	.79	.81	.00	.00	.21	.19
E*	.75	.80	.00	.00	.25	.20
F	.00	.06	.05	.20	.94	.74
G*	.89	.78	.00	.00	.11	.22
H*	.99	.90	.00	.00	.01	.10
J	.00	.00	.00	.00	1.00	.00
K*	.92	.83	.00	.00	.08	.17
L	.59	.72	.00	.00	.41	.29
M	.05	.68	.04	.11	.91	.74

PMPc = Posterior Model Probabilities of H0, H1, and the complement H2; GORICA weights = weights of H0, H1, and the unconstrained hypothesis Hu.

*Informative hypothesis is supported.

a sudden drop in gained body awareness (Patient F and M) or an increase in body dissociation instead of the expected decrease (Patient J). For patient L, the analyses were inconclusive, with the results equally favouring the informative and complement hypothesis. As can be seen from the individual plot, scores of body dissociation fluctuated over time, perhaps explaining the inconclusive results. Finally, in line with the individual plot of patient B, no changes occurred during treatment, and analyses preferred the null hypotheses. Note that all these Bayesian results are corroborated by the GORICA weights, which show the method robustness of the outcomes.

Therapists' evaluation of the module (aim 4)

Two major themes arose from the group discussion held with therapists ($n = 5$): “potential working mechanisms in reducing sexual revictimisation risk” and “changes attributed to the module”.

Regarding the working mechanisms of the module, therapists appreciated the interactive exercises (e.g. role-play); they believed it built rapport, allowed for active participation and facilitated integration in daily life. It was noted that for survivors of sexual violence, what feels familiar or comfortable might not be safe. Therefore, inviting patients to step slightly out of their comfort zone and practice new behaviour was considered an essential element of the module. Gendered behavioural patterns, such as compliance, keeping quiet, and considering others before oneself, came up during sessions and were considered key topics.

Therapists struggled with introducing the risk of “sexual revictimisation” without stepping into “victim-blaming”. Some therapists preferred being upfront about the risks to exonerate the patient, provide a sense of control, and align with the patient’s own fears. One therapist opposed “I want to show them the world is a safe place, and this message goes against that ... I feel I like am making their world unsafe”. Others felt reluctant conveying “If you behave differently, you can prevent victimisation” and questioned if it could be perceived as placing blame.

Furthermore, therapists recognised the complexity in addressing sexual revictimisation. They agreed that patients' risk of revictimisation is often embedded in early developmental patterns and closely linked to contextual factors. They expressed concerns about the inability of the module to address such long-standing patterns and social circumstances. Nevertheless, therapists argued that the module could still "plant a seed" or "turn on the volume of intuition" when evaluating potentially harmful situations or recognising safety. More firmly phrased, one therapist said "It is our duty to reduce such risks, when we are able to. We need to stay away from fatalistic thinking, thinking it is so complex that we should not even try to address it".

Therapists attributed various changes to the module: an increased awareness of one's emotions or bodily sensations, recognising and acting on signals of (un)safety, a greater sense of control over one's (negative) feelings or life, having a wider range of behavioural strategies at one's disposal, (re-)gained self-confidence, voicing boundaries, and being able to feel closer to those around them. Concerning clinical observations of change, therapists noticed gained confidence, inner strength, seeing someone prospers, or taking on new body postures. Overall, therapists noted that integrating gained insights or skills into a patient's life seemed to last. Therapists based these findings on what was shared with them by the patients over the course of treatment and thereafter.

Fictitious case

This case is compiled from the collective experiences of therapists but does not reflect one patient in particular.

Ibrahim is a 30-year-old man who identifies as homosexual; he was stigmatised by his community from a young age on. He experienced several occurrences of sexual violence and was in a relationship where he was sexually exploited. Often, he would comply with any sexual advances to ensure it would be over quickly and to avoid anger towards him. He wanted to learn how to set boundaries when someone initiated sexual contact. On his way to a treatment appointment, he was approached in the metro by someone offering him money for sex. He was taken aback by this and wondered why this person approached him specifically. Ibrahim and his therapist decided to integrate this event into the "controlled approach exercise". During the exercise, the therapist slowly walked towards Ibrahim, with him being in charge of communicating the distance that felt comfortable and safe to him. He decided to use the word "no" to communicate his boundary and practiced aligning his "no" with his tone of voice and body posture. The exercise was difficult for him; he felt as if he was yelling at the therapist, was worried about harming the therapeutic relationship, and reported feelings of tension. The therapist reassured Ibrahim. During the next session, he reflected back on the exercise, saying he never considered saying "no" to be an option in interacting with others. Further, along in the module, he entered the meeting in a cheerful mood. He told the therapist a friend (sexual partner) texted him to come over late at night; he did not feel like it; he was unsure about himself and felt a lot of tension. However, he finally texted back, saying he would rather not meet tonight, and the friend texted back, saying "No problem". This success, although seemingly minor, was a major turning point for Ibrahim and a realisation that his "no" could indeed be heard and respected.

Discussion

In this study, a novel body-oriented treatment module aimed at sexual revictimisation risk mitigation was evaluated for its feasibility in a small sample of migrant survivors of sexual violence and trafficking. The main findings were that completion of the treatment was found to be acceptable, adherence rates were high, and both patients and therapists attributed changes to the module that were in line with the intended treatment outcomes. It can be preliminarily concluded that the module is feasible amongst migrants and its further implementation and effectiveness are worthwhile to consider. These findings are of particular interest given the limited literature available on addressing the sexual revictimisation risks among migrant populations. Furthermore, although the body-oriented approaches are promising, they are rarely validated for migrant populations (O'Brien & Charura, 2022; Schaeffer & Cornelius-White, 2021; Van De Kamp et al., 2023). Study findings may be seen as a first step in identifying underlying mechanisms for understanding and addressing sexual revictimisation risks. Although there is a vast body of the literature on the risk and potential mechanisms of sexual revictimisation, how to address this risk within mental healthcare has received less attention (Classen et al., 2005; Hébert et al., 2021; Ullman & Vasquez, 2015; Walker & Wamser-Nanney, 2023). Study findings are in line with the literature promoting the relevance of body awareness, body dissociation, and efficacy in communicating boundaries in mitigating sexual revictimisation risks (Bockers et al., 2014; Katz et al., 2010; Kelley et al., 2016; Risser et al., 2006; Walker & Wamser-Nanney, 2023). Patients expressed gained insight into their personal boundaries, body awareness, (un)safe situations, sense of control, and connection to others. Moreover, analyses revealed that in seven out of 12 patients, body awareness/connection, as well as efficacy in communicating boundaries, were altered in accordance with the module's aims. As the fictitious case illustrates, the risk of sexual revictimisation may be high, and the module may bring awareness in recognizing and expressing boundaries.

Limitations

This study has several limitations that need to be considered. First, conclusions about the effectiveness and generalisability of the module cannot be drawn, given the uncontrolled design of the study and its small sample size. Second, patients were only followed for a short time. Therefore, no inference can be made about the actual mitigation of sexual revictimisation risks amongst the study sample, merely of its hypothesised underlying psychological mechanisms. Third, the use of single-item measures in this study may have compromised the depth and reliability needed to adequately capture complex constructs, potentially oversimplifying the nuanced effects of the treatment. A final consideration is that the developers of the module were closely engaged during each phase of the study, e.g. training, data collection, and monitoring of the treatment process. Although the researchers acknowledged this and may be seen as a strength, this level of involvement might also have favourably influenced the commitment of therapists or evoked preferred answers during evaluations and discussions.

Implications for clinical practice

Findings underline the relevance and feasibility of addressing sexual revictimisation among survivors of sexual violence and trafficking and may encourage therapists to incorporate this urgent topic into clinical practice. Notably, while developing and delivering the module, several adjustments were made, to tailor the structure and content to the daily reality of migrant survivors. First, a short-term module was implemented, given the difficulty this population often experiences in committing to long-term therapies (Semmlinger & Ehring, 2022). Nonetheless, for some patients, participation in the module was interrupted by challenges in balancing motherhood and therapy or due to changes in their asylum procedures. These practical considerations remain a challenge and warrant attention for future implementation. Additionally, therapists observed that cross-cultural discussions on gendered behavioural patterns were particularly insightful for patients. In line with this, session vignettes were modified to reflect each patient's daily life and personal beliefs.

The reflections shared by both patients and therapists provide valuable guidance for potential adaptations when delivering the module. Many patients emphasised the importance of reconnecting with their emotions, such as allowing themselves to feel rather than avoid them should remain central to the module's delivery. Therapists also noted the need to balance between challenging patients and respecting their emotional boundaries. Several elements of the module were disliked by the patients, like the controlled approach or grounded standing exercises, and therapists noted that the module could be burdensome for patients at times. However, therapists also indicated that for the module to be effective, patients might have to step out of their comfort zones or familiarities at times. Gradual exposure to such exercises or uncomfortable topics can encourage deeper engagement without overwhelming patients.

Therapists stressed the importance of introducing sexual revictimisation risks carefully, ensuring patients feel empowered rather than blamed. Discussions should focus on increasing self-awareness and promoting agency in potentially harmful situations, framing these conversations as opportunities to enhance safety without implying that victimisation is entirely within the patient's control. The study revealed that while some patients improved in body awareness, dissociation, and self-efficacy, others struggled with fluctuations or minimal change. The module's exercises, which allow for practicing and integrating skills in real-world situations, may be crucial to retain these improvements throughout the treatment consistently. This highlights the need for a tailored approach to treatment to sustain progress beyond therapy and ensure lasting impact.

Therapists planning to implement this module amongst survivors of sexual violence may consider the timing of offering the module as well. It is recommended to offer the module only in addition to evidence-based guideline-recommended therapies and only after partaking in trauma-focussed therapy and when patients are no longer hindered by triggers or intrusive memories of their sexual traumas.

Research implications

Next to scaling up the implementation of the module, future studies may consider longitudinal and controlled research within larger and more diverse samples on the

underlying mechanisms of sexual revictimisation (see for instance, Hébert et al., 2021; Widom, 2024). This would allow further understanding and substantiating potential underlying mechanisms of sexual exploitation targeted within the module, including their assumed contribution to actual sexual revictimisation risk mitigation. Clearly, in the present study, not all survivors were completers or responded to the module's intended outcomes. Future research could shed light on how the module could be tailored to the specific needs of various migrant groups, e.g. diverse SOGIE (Martinez & Kelle, 2013) or those who were in an intimate relationship with the perpetrator (Casassa et al., 2022). Also, this study revealed mixed findings concerning the direction of change in outcome measures during treatment. Future research could focus on elucidating the underlying reasons behind these varying patient outcomes. Future studies may aim to triangulate these quantitative findings with qualitative reflections from both therapists' and patients' perspectives.

The present study may be seen as a preliminary exploration of how to understand and address sexual revictimisation among migrant populations. Currently, mental health care for migrant sexual violence survivors rarely focusses on sexual revictimisation risk mitigation. As a body-oriented approach was found to be feasible, this might encourage mental health professionals to address sexual revictimisation, whilst acknowledging it is multifaceted and influenced by various contextual factors (e.g. Golo & Eshun, 2019; Levine & Schumacher, 2017; Spangaro et al., 2021).

Acknowledgments

We thank all patients and facilitators for their time and willingness to participate in this novel module. We thank Linda Verhaak, Linda Spanjaards, Naomi Namutebe, and Erika Buijze for their efforts during various stages of the development of the module. We gratefully acknowledge the support of Prof. Herbert Hoijtink in the analyses. We thank students Marielle de Goede, Jason Nijhuis, and Janneke Zoeten for their support in data collection and management.

Disclosure statement

No potential conflict of interest was reported by the author(s).

Funding

RG received partial funding for this project from an anonymous private fund. The study protocol did not undergo peer review by this fund. The funder will not have the ultimate authority over any study activities.

Notes on contributors

Rina Ghaforkhan is a senior researcher and policy adviser at ARQ National Psychotrauma Centre. She obtained her PhD at the Department of Clinical Psychology at Utrecht University on improving the mental healthcare for forced migrants and survivors of sexual violence. She has worked for various humanitarian aid organizations in (post-)conflict settings and protracted displacement settings on improving mental healthcare and psychosocial support.

Willem Scholte is a psychiatrist and co-founder of Laguna Collective. He gained broad international experience in his work with various humanitarian aid organizations in (post-)conflict and refugee settings. He holds a PhD in mental health in war-affected populations. He has been affiliated with the Amsterdam University Medical Centers (AMC) since the start of his career.

Saskia Bieleveldt is a psychomotor and sensorimotor therapist at Adagio Amsterdam. Before Adagio, she worked for 20 years at ARQ Centrum'45, where she worked with refugees, people with professional-related trauma, like veterans and police officers, and survivors of sexual and gender-based violence, sex trafficking, and displacement. She has a special interest in recovering intimacy and sexuality after experiences of sexual violence.

Niels van der Aa was a senior researcher and data specialist at ARQ National Psychotrauma Centre for over a decade. He currently works as an Assistant Professor at the Academic Centre for Dentistry Amsterdam.

Paul A. Boelen is a full professor in the Department of Clinical Psychology at Utrecht University and psychotherapist at ARQ Centrum'45. Paul Boelen is a licensed supervisor and cognitive-behavioral therapist with the Dutch Association for Cognitive and Behavioural Therapy and is also affiliated with the ARQ National Psychotrauma Centre.

Author contributions

RG: conceptualisation, methodology, formal analysis, investigation, writing – original draft, project administration, and funding acquisition.

WS: conceptualisation, methodology, supervision, writing – review and editing, funding acquisition.

SB: conceptualisation, methodology, writing – review and editing

NvdA: methodology, formal analysis, supervision, writing – original draft, review and editing.

PB: conceptualisation, methodology, supervision, writing – review and editing.

Data availability statement

The datasets generated and/or analysed for the current study are not publicly available due to the confidentiality of the patient data. The data that support the findings of this study are available from the corresponding author upon reasonable request.

References

- Abas, M., Ostrovski, N. V., Prince, M., Gorceag, V. I., Trigub, C., & Oram, S. (2013). Risk factors for mental disorders in women survivors of human trafficking: A historical cohort study. *BMC Psychiatry*, 13(1), 204. <https://doi.org/10.1186/1471-244X-13-204>
- Altinisik, Y., Van Lissa, C. J., Hoijtink, H., Oldehinkel, A. J., & Kuiper, R. M. (2021). Evaluation of inequality constrained hypotheses using a generalization of the AIC. *Psychological Methods*, 26(5), 599–621. <https://doi.org/10.1037/met0000406>
- Baba, M., Tipton, C., Danailova-Trainor, G., Morgan, K., & Matthew, R. (2023). Understanding re-trafficking in the United States: Why do we have a revolving door? In M. D. Chesnay & D. Sabella (Eds.), *Human trafficking: A global health emergency* (pp. 149–175). Springer International Publishing. https://doi.org/10.1007/978-3-031-33875-5_11
- Bhuptani, P. H., & Messman-Moore, T. L. (2019). Blame and shame in sexual assault. In W. T. O'Donohue & P. A. Schewe (Eds.), *Handbook of sexual assault and sexual assault prevention* (pp. 309–322). Springer International Publishing. https://doi.org/10.1007/978-3-030-23645-8_18

- Blackmore, R., Boyle, J. A., Fazel, M., Ranasingha, S., Gray, K. M., Fitzgerald, G., Misso, M., Gibson-Helm, M., & Spiegel, P. (2020). The prevalence of mental illness in refugees and asylum seekers: A systematic review and meta-analysis. *PLOS Medicine*, 17(9), e1003337. <https://doi.org/10.1371/journal.pmed.1003337>
- Bockers, E., Roepke, S., Michael, L., Renneberg, B., Knaevelsrud, C., & Brennan, C. H. (2014). Risk recognition, attachment anxiety, self-efficacy, and state dissociation predict revictimization. *PLoS One*, 9(9), e108206. <https://doi.org/10.1371/journal.pone.0108206>
- Boskovic, M., & Jankovic, B. (2023). Forced migrations and the risk of human trafficking. In E. Alaverdov & M. W. Bari (Eds.), *Advances in public policy and administration* (pp. 18–36). IGI Global. <https://doi.org/10.4018/978-1-6684-6334-5.ch002>
- Braun, V., & Clarke, V. (2022). *Thematic analysis: A practical guide*. SAGE.
- Breitenbecher, K. H. (2001). Sexual revictimization among women. *Aggression & Violent Behavior*, 6(4), 415–432. [https://doi.org/10.1016/S1359-1789\(00\)00014-8](https://doi.org/10.1016/S1359-1789(00)00014-8)
- Bremner, J. D., Bolus, R., & Mayer, E. A. (2007). Psychometric properties of the early trauma inventory–self report. *Journal of Nervous & Mental Disease*, 195(3), 211–218. <https://doi.org/10.1097/01.nmd.0000243824.84651.6c>
- Buckinx, B., Lata Hogg, C., & Vaughn, L. (2022). *The intersection of conflict-related sexual violence against men, boys, and LGBTQI+ persons and human trafficking*. Liechtenstein Institute on Self-Determination, Princeton University. https://oar.princeton.edu/bitstream/88435/pr10r9m39z/1/1222_CRSV.pdf
- Burback, L., Brémault-Phillips, S., Nijdam, M. J., McFarlane, A., & Vermetten, E. (2023). Treatment of posttraumatic stress disorder: A state-of-the-art review. *Current neuropharmacology*, 21(4), 557–635. <https://doi.org/10.2174/1570159X21666230428091433>
- Buss, D. E. (2009). Rethinking ‘rape’ as a weapon of war. *Feminist Legal Studies*, 17(2), 145–163. <https://doi.org/10.1007/s10691-009-9118-5>
- Byrow, Y., Pajak, R., Specker, P., & Nickerson, A. (2020). Perceptions of mental health and perceived barriers to mental health help-seeking amongst refugees: A systematic review. *Clinical Psychology Review*, 75, 101812. <https://doi.org/10.1016/j.cpr.2019.101812>
- Casassa, K., Knight, L., & Mengo, C. (2022). Trauma bonding perspectives from service providers and survivors of sex trafficking: A scoping review. *Trauma, Violence & Abuse*, 23(3), 969–984. <https://doi.org/10.1177/1524838020985542>
- Classen, C. C., Palesh, O. G., & Aggarwal, R. (2005). Sexual revictimization: A review of the empirical literature. *Trauma, Violence & Abuse*, 6(2), 103–129. <https://doi.org/10.1177/1524838005275087>
- David, F., Bryant, K., & Larsen, J. J. (2019). *Migrants and their vulnerability to human trafficking, modern slavery, and forced labor*. International Organization for Migration. <https://publications.iom.int>
- De Schrijver, L., Vander Beken, T., Krahé, B., & Keygnaert, I. (2018). Prevalence of sexual violence in migrants, applicants for international protection, and refugees in Europe: A critical interpretive synthesis of the evidence. *International Journal of Environmental Research and Public Health*, 15(9), 1979. <https://doi.org/10.3390/ijerph15091979>
- Downing, N. R., Akinlotan, M., & Thornhill, C. W. (2021). The impact of childhood sexual abuse and adverse childhood experiences on adult health related quality of life. *Child Abuse and Neglect*, 120, 105181. <https://doi.org/10.1016/j.chiabu.2021.105181>
- Dworkin, E. R. (2020). Risk for mental disorders associated with sexual assault: A meta-analysis. *Trauma, Violence & Abuse*, 21(5), 1011–1028. <https://doi.org/10.1177/1524838018813198>
- Dworkin, E. R., Jaffe, A. E., Bedard-Gilligan, M., & Fitzpatrick, S. (2023). PTSD in the year following sexual assault: A meta-analysis of prospective studies. *Trauma, Violence & Abuse*, 24(2), 497–514. <https://doi.org/10.1177/15248380211032213>
- Farrell, J. M., & Shaw, I. A. (1994). Emotional awareness training: A prerequisite to effective cognitive-behavioral treatment of borderline personality disorder. *Cognitive and Behavioral Practice*, 1(1), 71–91. [https://doi.org/10.1016/S1077-7229\(05\)80087-2](https://doi.org/10.1016/S1077-7229(05)80087-2)
- Flanders, C. E., Tarasoff, L. A., & VanKim, N. (2023). Sexual violence and mental health among young bi+ and lesbian women and gender minoritized people. *Journal of Gay & Lesbian Mental Health*, 27(4), 483–502. <https://doi.org/10.1080/19359705.2022.2072036>

- Foundation, P. (2019). *Handbook: Holistic care for survivors of sexual violence in conflict*. https://mukwegefoundation.org/wp-content/uploads/2019/07/Handbook-Care_for_SV_Survivors-ENG-screen.pdf
- García-Vázquez, O., & Meneses-Falcón, C. (2023). What is the impact of human trafficking on the biopsychosocial health of victims: A systematic review. *Journal of Immigrant & Minority Health*, 26(1), 148–162. <https://doi.org/10.1007/s10903-023-01496-z>
- Golo, H. K., & Eshun, I. (2019). 'Re-trafficking' in the coastal communities and the Volta Lake of Ghana: Children's rights, agency and intra-household bargaining position. *American Journal of Business and Society*, 4(4), 97–107.
- Gu, X., Hoijtink, H., Mulder, J., & van Lissa, C. (2021). *Bain: Bayes factors for informative hypotheses. (version 0.2.8). R package*. <https://CRAN.R-project.org/package=bain>
- Hébert, M., Lapierre, A., MacIntosh, H. B., & Ménard, A. D. (2021). A review of mediators in the association between child sexual abuse and revictimization in romantic relationships. *Journal of Child Sexual Abuse*, 30(4), 385–406. <https://doi.org/10.1080/10538712.2020.1801936>
- Henkelmann, J.-R., De Best, S., Deckers, C., Jensen, K., Shahab, M., Elzinga, B., & Molendijk, M. (2020). Anxiety, depression and post-traumatic stress disorder in refugees resettling in high-income countries: Systematic review and meta-analysis. *British Journal of Psychiatry Open*, 6(4), e68. <https://doi.org/10.1192/bjo.2020.54>
- Hoijtink, H. (2012). *Informative hypotheses: Theory and practice for behavioral and social scientists*. Chapman and Hall/CRC.
- Hoijtink, H., Mulder, J., Van Lissa, C., & Gu, X. (2019). A tutorial on testing hypotheses using the Bayes factor. *Psychological Methods*, 24(5), 539–556. <https://doi.org/10.1037/met0000201>
- International Organization for Migration. (2019). *Mapping report on legal frameworks and assistance available to migrant victims of sexual and gender-based violence (SGBV)*. <https://iom-nederland.nl/images/Protect/PROTECT-project-mapping-legal-framework-SGBV.pdf>
- Katz, J., May, P., Sörensen, S., & DelTosta, J. (2010). Sexual revictimization during women's first year of college: Self-blame and sexual refusal assertiveness as possible mechanisms. *Journal of Interpersonal Violence*, 25(11), 2113–2126. <https://doi.org/10.1177/0886260509354515>
- Kearney, B. E., & Lanius, R. A. (2022). The brain-body disconnect: A somatic sensory basis for trauma-related disorders. *Frontiers in Neuroscience*, 16, 1015749. <https://doi.org/10.3389/fnins.2022.1015749>
- Kelley, E. L., Orchowski, L. M., & Gidycz, C. A. (2016). Sexual victimization among college women: Role of sexual assertiveness and resistance variables. *Psychology of Violence*, 6(2), 243–252. <https://doi.org/10.1037/a0039407>
- Keygnaert, I., Vettenburg, N., & Temmerman, M. (2012). Hidden violence is silent rape: Sexual and gender-based violence in refugees, asylum seekers and undocumented migrants in Belgium and the Netherlands. *Culture, Health & Sexuality*, 14(5), 505–520. <https://doi.org/10.1080/13691058.2012.671961>
- Latham-Sprinkle, J., David, F., Bryant, K., & Larsen, J. (2019). *Migrants and their vulnerability to human trafficking, modern slavery and forced labour*. <http://hdl.handle.net/1854/LU-8636089>
- Levine, J. A., & Schumacher, U. (2017). Mental health issues in survivors of sex trafficking. *Cogent Medicine*, 4(1), 1278841. <https://doi.org/10.1080/2331205X.2017.1278841>
- Li, S. S. Y., Liddell, B. J., & Nickerson, A. (2016). The relationship between post-migration stress and psychological disorders in refugees and asylum seekers. *Current Psychiatry Reports*, 18(9), 82. <https://doi.org/10.1007/s11920-016-0723-0>
- Littleton, H., & Decker, M. (2017). Predictors of resistance self-efficacy among rape victims and association with revictimization risk: A longitudinal study. *Psychology of Violence*, 7(4), 583–592. <https://doi.org/10.1037/vio0000066>
- Livingston, J. A., Testa, M., & VanZile-Tamsen, C. (2007). The reciprocal relationship between sexual victimization and sexual assertiveness. *Violence Against Women*, 13(3), 298–313. <https://doi.org/10.1177/1077801206297339>
- Macy, R. J. (2007). A coping theory framework toward preventing sexual revictimization. *Aggression & Violent Behavior*, 12(2), 177–192. <https://doi.org/10.1016/j.avb.2006.09.002>

- Maercker, A., Cloitre, M., Bachem, R., Schlumpf, Y. R., Khoury, B., Hitchcock, C., & Bohus, M. (2022). Complex post-traumatic stress disorder. *Lancet*, 400(10345), 60–72. [https://doi.org/10.1016/S0140-6736\(22\)00821-2](https://doi.org/10.1016/S0140-6736(22)00821-2)
- Martinez, O., & Kelle, G. (2013). Sex trafficking of LGBT individuals: A call for service provision, research, and action. *The International Law News*, 42(4).
- Messman-Moore, T. L., & Brown, A. L. (2006). Risk perception, rape, and sexual revictimization: A prospective study of college women. *Psychology of Women Quarterly*, 30(2), 159–172. <https://doi.org/10.1111/j.1471-6402.2006.00279.x>
- Messman-Moore, T. L., & McConnell, A. A. (2018). Intervention for sexual revictimization among college women. In *Sexual assault risk reduction and resistance* (pp. 309–330). Elsevier. <https://doi.org/10.1016/B978-0-12-805389-8.00013-X>
- Mills, B., & Turnbull, G. (2004). Broken hearts and mending bodies: The impact of trauma on intimacy. *Sexual & Relationship Therapy*, 19(3), 265–289. <https://doi.org/10.1080/14681990410001715418>
- National Rapporteur on Trafficking in Human Beings and Sexual Violence against Children. (2021). *Slachtoffermonitor mensenhandel 2016-2020 [Survivor monitor human trafficking 2016-2020]*. Nationaal Rapporteur.
- Neuner, F. (2023). Physical and social trauma: Towards an integrative transdiagnostic perspective on psychological trauma that involves threats to status and belonging. *Clinical Psychology Review*, 99, 102219. <https://doi.org/10.1016/j.cpr.2022.102219>
- Nilsson, J. E., Jeevanba, S. B., Molitoris, A., Stratmann, S., & Kubik, R. (2021). Sexual and gender minority refugees and asylum seekers: An arduous journey. In E. M. L. In, C. Burgess, & A. J. Johnson (Eds.), *Violence against LGBTQ+ persons* (pp. 231–239). Springer International Publishing. https://doi.org/10.1007/978-3-030-52612-2_18
- Noll, J. G., & Grych, J. H. (2011). Read-react-respond: An integrative model for understanding sexual revictimization. *Psychology of Violence*, 1(3), 202–215. <https://doi.org/10.1037/a0023962>
- O'Brien, C. V., & Charura, D. (2022). Refugees, asylum seekers, and practitioners' perspectives of embodied trauma: A comprehensive scoping review. *Psychological Trauma: Theory, Research, Practice, & Policy*, 15(7), 1115–1127. <https://doi.org/10.1037/tra0001342>
- Ogden, P. (2015). *Sensorimotor psychotherapy: Interventions for trauma and attachment* (First ed.). W. W. Norton & Company.
- Oram, S. (2019). Sexual violence and mental health. *Epidemiology and Psychiatric Sciences*, 28(6), 592–593. <https://doi.org/10.1017/S2045796019000106>
- Ottisova, L., Hemmings, S., Howard, L. M., Zimmerman, C., & Oram, S. (2016). Prevalence and risk of violence and the mental, physical and sexual health problems associated with human trafficking: An updated systematic review. *Epidemiology and Psychiatric Sciences*, 25(4), 317–341. <https://doi.org/10.1017/S2045796016000135>
- Price, C. J., & Thompson, E. A. (2007). Measuring dimensions of body connection: Body awareness and bodily dissociation. *The Journal of Alternative and Complementary Medicine*, 13(9), 945–953. <https://doi.org/10.1089/acm.2007.0537>
- Price, C. J., Thompson, E. A., Cheng, S. C., & Aguilera, A. I. (2017). Scale of body connection: A multi-sample construct validation study. *PLoS One*, 12(10), e0184757. <https://doi.org/10.1371/journal.pone.0184757>
- Rich, C. L., Combs-Lane, A. M., Resnick, H. S., & Kilpatrick, D. G. (2004). Child sexual abuse and adult sexual revictimization. In L. J. Koenig, L. S. Doll, A. O'Leary, & W. Pequegnat (Eds.), *From child sexual abuse to adult sexual risk: Trauma, revictimization, and intervention* (pp. 49–68). American Psychological Association. <https://doi.org/10.1037/10785-003>
- Risser, H. J., Hetzel-Riggin, M. D., Thomsen, C. J., & McCanne, T. R. (2006). PTSD as a mediator of sexual revictimization: The role of reexperiencing, avoidance, and arousal symptoms. *Journal of Traumatic Stress*, 19(5), 687–698. <https://doi.org/10.1002/jts.20156>
- Robitz, R., Asera, A., Nguyen, P., Gordon, M., Coverdale, J., Stoklosa, H., & Chisolm-Straker, M. (2022). An annotated bibliography on human trafficking for the mental health clinician. *Journal of Psychiatric Practice*, 28(3), 218–226. <https://doi.org/10.1097/PRA.0000000000000636>
- Rothman, K., Georgia Salivar, E., Roddy, M. K., Hatch, S. G., & Doss, B. D. (2021). Sexual assault among women in college: Immediate and long-term associations with mental health, psychosocial

- functioning, and romantic relationships. *Journal of Interpersonal Violence*, 36(19–20), 9600–9622. <https://doi.org/10.1177/0886260519870158>
- Rubini, E., Valente, M., Trentin, M., Facci, G., Ragazzoni, L., & Gino, S. (2023). Negative consequences of conflict-related sexual violence on survivors: A systematic review of qualitative evidence. *International Journal for Equity in Health*, 22(1), 227. <https://doi.org/10.1186/s12939-023-02038-7>
- Schaeffer, A. J., & Cornelius-White, J. H. D. (2021). Qualitative studies on body-based interventions for refugees: A meta-synthesis. *Body, Movement and Dance in Psychotherapy*, 16(4), 267–285. <https://doi.org/10.1080/17432979.2021.1893810>
- Schauer, M., & Elbert, T. (2010). Dissociation following traumatic stress: Etiology and treatment. *Zeitschrift für Psychologie/Journal of Psychology*, 218(2), 109–127. <https://doi.org/10.1027/0044-3409/a000018>
- Schimmenti, A., & Caretti, V. (2016). Linking the overwhelming with the unbearable: Developmental trauma, dissociation, and the disconnected self. *Psychoanalytic Psychology*, 33(1), 106–128. <https://doi.org/10.1037/a0038019>
- Scholte, W. F., & Ager, A. K. (2014). Social capital and mental health: Connections and complexities in contexts of post conflict recovery. *Intervention*, 12(2), 210. <https://doi.org/10.1097/WTF.0000000000000038>
- Schulz, K. F., Altman, D. G., & Moher, D. (2010). CONSORT 2010 statement: Updated guidelines for reporting parallel group randomised trials. *Journal of Pharmacology & Pharmacotherapeutics*, 1(2), 100–107. <https://doi.org/10.4103/0976-500X.72352>
- Scoglio, A. A. J., Kraus, S. W., Saczynski, J., Joorna, S., & Molnar, B. E. (2021). Systematic review of risk and protective factors for revictimization after child sexual abuse. *Trauma, Violence & Abuse*, 22(1), 41–53. <https://doi.org/10.1177/1524838018823274>
- Semmlinger, V., & Ehring, T. (2022). Predicting and preventing dropout in research, assessment and treatment with refugees. *Clinical Psychology & Psychotherapy*, 29(3), 767–782. <https://doi.org/10.1002/cpp.2672>
- Spangaro, J., Toole-Anstey, C., MacPhail, C. L., Rambaldini-Gooding, D. C., Keevers, L., & Garcia-Moreno, C. (2021). The impact of interventions to reduce risk and incidence of intimate partner violence and sexual violence in conflict and post-conflict states and other humanitarian crises in low and middle income countries: A systematic review. *Conflict and Health*, 15(1), 86. <https://doi.org/10.1186/s13031-021-00417-x>
- Tankink, M., & Richters, A. (2007). Silence as a coping strategy: The case of refugee women in the Netherlands from South-Sudan who experienced sexual violence in the context of war. In B. Droždek & J. P. Wilson (Eds.), *Voices of trauma: Treating psychological trauma across cultures* (pp. 191–210). Springer US.
- Tol, W. A., Stavrou, V., Greene, M. C., Mergenthaler, C., Van Ommeren, M., & García Moreno, C. (2013). Sexual and gender-based violence in areas of armed conflict: A systematic review of mental health and psychosocial support interventions. *Conflict and Health*, 7(1), 16. <https://doi.org/10.1186/1752-1505-7-16>
- Ullman, S. E., & Vasquez, A. L. (2015). Mediators of sexual revictimization risk in adult sexual assault victims. *Journal of Child Sexual Abuse*, 24(3), 300–314. <https://doi.org/10.1080/10538712.2015.1006748>
- United Nations High Commissioner for Refugees. (2023). *Global trends forced displacement in 2022*. <https://www.unhcr.org/global-trends-report-2022>
- U.S. Department of Veterans Affairs. (2022). *PTSD checklist for DSM-5*. (PCL–5). <https://www.ptsd.va.gov/professional/assessment/adult-sr/ptsd-checklist.asp>
- Van Brabant, L., Kuiper, R., Rosseel, Y., & Dacko, A. (2023). *Restriktor*. CRAN - Package restriktor (*r-project.Org*).
- Van De Kamp, M. M., Scheffers, M., Emck, C., Fokker, T. J., Hatzmann, J., Cuijpers, P., & Beek, P. J. (2023). Body-and movement-oriented interventions for posttraumatic stress disorder: An updated systematic review and meta-analysis. *Journal of Traumatic Stress*, 36(5), 835–848. <https://doi.org/10.1002/jts.22968>
- Van der Kolk, B. A. (2015). *The body keeps the score: Mind, brain and body in the transformation of trauma*. Penguin Books.

- Van der Maas, L. C., Köke, A., Bosscher, R. J., Hoekstra, T., & Peters, M. L. (2014). *Measuring body awareness with the scale of body connection: Structure and reliability of the Dutch translation* [[PhD-Thesis –Research external, graduation internal, Vrije Universiteit Amsterdam.
- Van Praag, D. L. G., Fardzadeh, H. E., Covic, A., Maas, A. I. R., Von Steinbüchel, N., & Tran, T. D. (2020). Preliminary validation of the Dutch version of the posttraumatic stress disorder checklist for DSM-5 (PCL-5) after traumatic brain injury in a civilian population. *PLoS One*, 15(4), e0231857. <https://doi.org/10.1371/journal.pone.0231857>
- Vu, A., Adam, A., Wirtz, A., Pham, K., Rubenstein, L., Glass, N., Beyrer, C., & Singh, S. (2014). The prevalence of sexual violence among female refugees in complex humanitarian emergencies: A systematic review and meta-analysis. *PLoS Currents*. <https://doi.org/10.1371/currents.dis.835f10778fd80ae031aac12d3b533ca7>
- Walker, H. E., Freud, J. S., Ellis, R. A., Fraine, S. M., & Wilson, L. C. (2019). The prevalence of sexual revictimization: A meta-analytic review. *Trauma, Violence & Abuse*, 20(1), 67–80. <https://doi.org/10.1177/1524838017692364>
- Walker, H. E., & Wamser-Nanney, R. (2023). Revictimization risk factors following childhood maltreatment: A literature review. *Trauma, Violence & Abuse*, 24(4), 2319–2332. <https://doi.org/10.1177/15248380221093692>
- Widom, C. S. (2024). Childhood maltreatment, revictimization, and partner violence victimization through midlife: A prospective longitudinal investigation. *Journal of Interpersonal Violence*, 39(19–20), 4087–4112. <https://doi.org/10.1177/08862605241264524>
- Wolfe, D. A. (Ed.). (1996). *The youth relationships manual: A group approach with adolescents for the prevention of woman abuse and the promotion of healthy relationships*. Sage Publications.